VUGA NDAKUMVA

MENTAL HEALTH EDUCATIONAL CONTENT

Chapter 07
Understanding Grief, Loss, and Emotional Adjustment

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Grief can be painful, confusing, and exhausting. It can also change over time, sometimes without warning. This chapter explains different kinds of loss, common reactions, ways to find support, and when professional assessment may help. It does not require readers to disclose personal losses or revisit difficult memories.

Educational safety statement: This chapter is general mental health education, not a diagnosis, psychotherapy, or an individual treatment plan. Readers may skip activities. Where there is immediate danger, seek urgent local medical or emergency assistance.

Learning outcomes

- Describe grief, bereavement, mourning, and emotional adjustment in plain language.
- Recognize emotional, physical, cognitive, social, and practical responses to loss.
- Understand why grieving does not follow a fixed set of stages or a universal timetable.
- Identify compassionate coping methods, helpful support, and ways to assist another person.
- Distinguish ordinary grief from situations needing assessment, including prolonged grief disorder.

07.1 — What grief means

Grief is the response people may have when someone or something important is lost. The loss may be the death of a loved one, the end of a relationship, leaving a familiar place, losing employment, a change in health, or the loss of an anticipated future. Responses can include sadness, longing, anger, guilt, relief, confusion, numbness, or even moments of happiness. None of these feelings alone defines how much the loss mattered.

Bereavement refers specifically to the experience of losing someone through death. Mourning is the way grief may be expressed within a family, culture, faith, or community, such as gatherings, remembrance, silence, or rituals. Adjustment means learning to live with changes that follow loss. Adjustment is not the same as forgetting or no longer caring. [1,2]

07.2 — Different types of loss

Death-related loss: A person may grieve a parent, partner, child, friend, colleague, or another important person. The relationship may have been loving, conflicted, distant, or complicated; each can produce genuine grief.

Living losses: The end of a relationship, migration, infertility, disability, an illness diagnosis, job loss, separation from a community, or a major change in personal identity can bring grief-like responses. These experiences deserve support, although the clinical diagnosis of prolonged grief disorder applies specifically to bereavement following death.

Anticipatory grief: Some people experience grief before a likely loss, such as when caring for a person with advanced illness. They may feel sadness and uncertainty while also hoping for improvement.

Ambiguous loss: Some losses have no clear ending, such as the unexplained absence of a loved one or major changes in a family member’s memory or personality. Uncertainty may make emotional adjustment particularly difficult. These terms describe experiences, not automatic diagnoses.

07.3 — Why everyone grieves differently

The meaning of the relationship, the circumstances of the loss, earlier experiences, physical health, family responsibilities, financial conditions, available support, and cultural expectations all affect grieving. One person may cry frequently; another may concentrate on practical tasks. Some seek company; others prefer periods of solitude. People may also move between these responses.

There is no requirement to experience grief in a certain order. Popular descriptions of “stages” may be useful language for some people, but they are not a universal sequence or a checklist that everyone must complete. [1]

07.4 — Emotional responses to grief

Sadness and longing may arise when a place, song, routine, or anniversary reminds a person of the loss. Anger may be directed at the situation, an institution, other people, or oneself. Guilt may involve repeated thoughts such as “I should have known” or “I could have done more.” Fear may concern finances, parenting alone, future relationships, or losing others.

Feeling numb, relieved, or temporarily happy does not mean the person is uncaring. Relief may coexist with sadness, particularly after a long period of caregiving or suffering. People may need permission to

hold several emotions at once without judging themselves.

07.5 — How grief can affect the body and daily life

Some bereaved people experience fatigue, interrupted sleep, appetite changes, headaches, muscle tension, difficulty concentrating, forgetfulness, low motivation, or reduced interest in social activities. Practical tasks can suddenly feel harder because attention and energy are limited.

Not every physical symptom is caused by grief. New, severe, or persistent medical symptoms should be assessed by a healthcare professional. Chest pain, significant breathing difficulty, fainting, or other emergency symptoms require urgent medical attention. [1]

07.6 — Grief over time: waves rather than a straight line

For many people, very intense feelings gradually become less constant, but reminders can bring renewed grief years later. A birthday, holiday, graduation, or family event may evoke sadness and affection together. Grief can change in character: a person may still miss someone deeply while becoming able to study, work, connect, and experience enjoyment.

The aim of adjustment is not to erase the relationship. A continuing sense of connection through memories, values, stories, and meaningful practices can coexist with taking part in present life. There is no deadline by which someone must “move on.”

07.7 — Grief, depression, and trauma: important distinctions

Grief often centres on the loss and reminders of it, with emotions that may rise and fall. Major depression may involve persistent low mood or loss of pleasure across many aspects of life, alongside other symptoms and impairment. PTSD may involve intrusive traumatic memories, avoidance, threat-related arousal, and other symptoms following traumatic exposure. These experiences can overlap; one person may require assessment for more than one condition.

Neither a short checklist nor the passage of a few weeks can establish a diagnosis. A professional evaluates the full pattern, medical factors, risk, functioning, and cultural context. [2,3]

07.8 — Prolonged grief disorder: when grief becomes severely impairing

A minority of bereaved people experience grief that remains unusually intense and pervasive, with strong yearning or preoccupation, substantial emotional pain, and major difficulty engaging in daily life. In prolonged grief disorder, the disturbance exceeds what is expected in the person’s cultural and social context and causes significant impairment.

The WHO ICD-11 describes grief persisting for an atypically long period, with more than six months as a minimum. The DSM-5-TR requires at least twelve months for adults and six months for children and adolescents, plus additional clinical symptoms and impairment. These thresholds are not self-diagnosis rules. Someone experiencing severe distress can seek support before either threshold. [3,4]

What may justify assessment

- Intense yearning or preoccupation that dominates much of everyday life and remains seriously impairing.
- Difficulty accepting the death, significant isolation, strong feelings of meaninglessness, or persistent difficulty resuming necessary activities.
- Persistent, distressing avoidance or preoccupation that appears to be restricting life.
- Major depression, trauma symptoms, risky substance use, or thoughts of self-harm accompanying grief.

07.9 — Healthy coping: meeting basic needs first

Grief can make ordinary routines seem demanding. Gentle attention to hydration, meals, rest, medication as prescribed, and safe physical movement may protect wellbeing. A person need not pursue a perfect routine. Small achievable steps, such as eating a meal or taking a brief walk, can be a reasonable beginning.

Try to avoid relying on alcohol, recreational drugs, or gambling to reduce grief-related pain. These may worsen mental health or create additional problems. If practical needs such as childcare, transport, paperwork, or finances are overwhelming, asking for concrete help can be as valuable as discussing feelings. [1,5]

07.10 — Coping with reminders, anniversaries, and memories

Reminders may arrive unexpectedly through photographs, songs, familiar places, smells, or conversations. If a reminder causes distress, a person may pause, notice their surroundings, and choose whether to continue, leave the situation, call someone, or rest. There is no obligation to view photos, keep particular belongings, or participate in rituals before feeling ready.

Anniversaries may be easier with a flexible plan: deciding how much company is wanted, arranging transport or a quiet space, choosing a meaningful activity if desired, and allowing plans to change. Some people prefer remembrance; others prefer ordinary routine. Both can be valid.

07.11 — Social support and asking for help

A supportive relationship does not require another person to fix grief. It may involve a listening ear, sitting together, helping with meals, accompanying someone to an appointment, caring for children, or checking in without demanding a response. Support should be offered rather than imposed.

People vary in whom they trust. Family, faith communities, peers, and community groups can be valuable when they respect privacy and consent, but no one should be pressured to disclose personal experiences or follow a particular mourning practice.

07.12 — Supporting someone who is grieving

Start with simple, direct care: “I’m sorry this has happened. I can listen if you want, or help with something practical.” Let the person decide whether to talk. Avoid comparing losses, insisting that they be strong, interpreting their emotions, or giving fixed deadlines.

Rather than saying “Let me know if you need anything,” offer a choice of specific assistance: preparing a meal, helping with transport, checking on household duties, or accompanying them to seek professional help. Follow through on promised support. [5]

Helpful and less-helpful responses

- Helpful: “Would you like to tell me about them, or would you prefer to sit quietly?”
- Helpful: “I can check in next week too, if that would be welcome.”
- Less helpful: “You should be over this by now.”
- Less helpful: “Everything happens for a reason,” unless the person has specifically welcomed that framing.

07.13 — Children and adolescents experiencing grief

Children may express grief through play, questions, clinginess, irritability, sleep changes, withdrawal, or changes at school. A young child may ask repeatedly about the death as their understanding develops. Adolescents may want privacy while still needing dependable adult support.

Give age-appropriate, truthful explanations; avoid confusing euphemisms when children need to understand that a person has died. Keep routines where possible and identify a trusted adult who can answer questions. Do not insist that children participate in memorial activities or share feelings publicly. Persistent changes in safety, behavior, sleep, or school functioning warrant professional assessment. [5]

07.14 — Grief across families and cultures

Communities differ in funeral practices, periods of mourning, how emotions are expressed, and how responsibility is shared. Rwanda’s diverse family and community contexts may shape remembrance, practical support, and use of faith traditions. A respectful educational resource asks what is meaningful to the individual rather than prescribing one response.

Some people find comfort in prayer or religious communities; others do not. Spiritual support may complement healthcare when freely chosen. No person should be blamed for grief, excluded from support, or told that clinical care is unnecessary because of beliefs.

07.15 — When to seek professional support

Consider speaking with a qualified health or mental health professional when grief is severely overwhelming, interferes with essential activities, continues to worsen, includes persistent depression or traumatic stress symptoms, or is accompanied by harmful substance use. Help is appropriate even when the loss happened recently.

A clinician can assess whether the person is experiencing a depressive disorder, PTSD, prolonged grief disorder, a physical health issue, or another concern, and discuss appropriate treatment and support. Some people benefit from structured grief-focused psychotherapy. Medication is not a universal treatment for grief itself; it may be considered for an independently diagnosed condition under professional guidance. [3,4,5]

07.16 — Safety and urgent help

Suicidal thoughts, intent to self-harm, inability to stay safe, severe confusion, or immediate threats to life require urgent help. Contact available local emergency services or go to the nearest emergency department. If a person is in immediate danger, remain with them only when safe to do so and seek urgent assistance.

If someone says they want to die, respond calmly and take the statement seriously. Do not argue, shame, or promise to keep imminent danger secret. Help connect them to qualified emergency or mental health care. The exact Rwanda referral numbers and facilities must be verified by VUGA NDAKUMVA before this chapter is published as a public-facing resource.

Part IV — Guided practical activities

Activity 1 — A small daily support check

Purpose: Notice basic needs without expecting grief to disappear. Time: 3–5 minutes. If useful, ask: Have I eaten or had water? Have I rested? Is there one simple task I can manage? Is there a safe person I might contact? Choose at most one small action.

Safety: This is not a recovery test. If the questions cause distress, stop. If basic care feels impossible for an extended period, seek help with practical and healthcare needs.

Activity 2 — Grounding during an intense wave

Purpose: Reconnect with the present when grief feels overwhelming. Time: 1–3 minutes. If comfortable, place your feet on the floor or notice the support of your seat; look around and name three ordinary objects; listen for two sounds; then decide on one safe next step, such as drinking water or calling someone.

Safety: Keep eyes open if preferred. Do not deliberately evoke painful memories. Stop if the activity worsens distress or causes feelings of disconnection. Grounding may help momentarily but is not a treatment for prolonged grief disorder or PTSD.

Activity 3 — A gentle support map

Purpose: Identify types of help rather than expecting one person to meet every need. On paper, optionally write: (1) a person who may listen; (2) someone who may help practically; (3) a health professional or local service; (4) a safe place or activity that offers a little relief. Leave any category blank if it is not available.

Safety: Do not include unsafe contacts, and keep the page private if disclosure might create problems. Reaching out is a choice, not a requirement.

Activity 4 — Remembering without pressure

Purpose: Give space to meaning and connection only if desired. Choose one low-intensity option: think of a value learned from the person, prepare a meal connected to a positive memory, or quietly reflect on something important in the relationship. Alternatively, skip memory work entirely and focus on present-day care.

Safety: Do not force detailed storytelling, trauma exposure, or visits to distressing sites. If memories become overwhelming, stop and seek support. Clinical grief treatment should be led by qualified practitioners.

Activity 5 — A manageable plan for a difficult date

Purpose: Reduce unnecessary pressure around an anniversary or important event. Decide: Where do I want to be? Do I want company? What basic tasks are essential? What will I do if I need a break? Which plans can I cancel?

Safety: Flexible plans are more useful than promises to “feel okay.” If the date triggers serious mental health symptoms, seek professional guidance.

Part V — Realistic learning scenarios

Scenario A — A student after a parent’s death

A university student returns to classes several weeks after a parent dies. They find concentration difficult and feel unexpectedly tearful when seeing classmates with their parents. They also worry about tuition and supporting younger siblings.

Educational response: The student’s reactions may be understandable in grief. Practical adjustments—such as asking the school about temporary academic flexibility, identifying a trusted person, and seeking financial support—may reduce pressure. If symptoms become severely impairing, professional support should be offered. No diagnosis should be inferred from this scenario.

Scenario B — A parent facing anniversary grief

A parent who lost a family member years ago finds that the anniversary is still painful. They can manage daily routines, experience enjoyment, and feel sadness when remembering.

Educational response: Continued sadness or memories do not automatically indicate a disorder. Planning a quiet activity or asking for company may help. A diagnosis would require assessment of distress, functioning, duration, and cultural context.

Scenario C — Loss of employment and identity

A worker loses employment after many years and feels embarrassed, angry, and uncertain about their role in the family. Their grief is linked to financial insecurity and an unexpected change in identity.

Educational response: Acknowledge the loss, provide practical support such as benefits or job-search assistance where available, and consider trusted social contact. The term prolonged grief disorder should not be applied to job loss; it concerns bereavement after death.

Scenario D — Severe grief that is not improving

More than a year after a partner’s death, an adult experiences near-constant yearning, significant isolation, difficulty meeting daily obligations, and a strong sense that life has no meaning.

Educational response: These experiences warrant a qualified mental health assessment, including questions about safety, mood, trauma, functioning, and cultural context. Prolonged grief disorder may be one consideration, but it cannot be diagnosed from this short example.

Scenario E — A child and confusing explanations

A child is told that a grandparent “went to sleep” and becomes afraid to go to bed. The child repeatedly asks where the grandparent has gone.

Educational response: A caregiver can give a brief, honest, developmentally appropriate explanation of death, allow questions, offer predictable routines, and involve a trained professional if distress is persistent or markedly affects functioning.

Part VI — Reflection and assessment

Optional reflection questions

- What does support mean to you at a difficult time?
- Which basic routines or practical tasks might need more support after a loss?
- How might cultural practices help one person but feel uncomfortable to another?
- What phrases could you use to listen without pressuring someone to speak?
- If severe distress persists, which qualified health professional or facility could you contact?

Do not require learners to describe their own bereavement. They may answer using a fictional situation or choose not to participate.

Knowledge check — with explained answers

1. Must grief occur through five fixed stages?

Answer: No. Grief experiences are variable, may come in waves, and do not follow a universally required order.

2. Can someone grieve a job or relationship loss?

Answer: Yes. Many important losses can produce grief-like feelings. The diagnosis of prolonged grief disorder, however, applies to bereavement after death.

3. Does being sad for six months automatically mean prolonged grief disorder?

Answer: No. Clinical diagnosis requires a fuller symptom pattern, significant impairment, cultural-context evaluation, and diagnostic timing criteria.

4. Should we ask a grieving person to recount the event in detail?

Answer: No. Listening without pressure and allowing choice are safer than forcing disclosure.

5. When is urgent help necessary?

Answer: When there is imminent danger, suicidal intent, inability to remain safe, or another medical or psychiatric emergency.

Essential glossary

- Grief — emotional, physical, social, and practical responses to an important loss.
- Bereavement — the experience of losing someone through death.
- Mourning — personal and social expressions of grief.
- Anticipatory grief — grief-like feelings before an expected loss.
- Prolonged grief disorder — a clinically recognized condition involving pervasive, impairing grief after bereavement, assessed in context.
- Emotional adjustment — adapting to changed circumstances and routines over time.

Part VII — Evidence and professional review

Foundational references (accessed 9 November 2026):

[1] National Health Service (NHS). Grief after bereavement or loss.
https://www.nhs.uk/mental-health/feelings-symptoms-behaviours/feelings-and-symptoms/grief-bereavement-loss/

[2] National Cancer Institute. Grief, Bereavement, and Coping With Loss (PDQ), Health Professional Version.
https://www.cancer.gov/about-cancer/advanced-cancer/caregivers/planning/bereavement-hp-pdq

[3] World Health Organization. ICD-11, 6B42 Prolonged grief disorder.
https://icd.who.int/dev11/l-m/en/GetConcept?ConceptId=http%3A%2F%2Fid.who.int%2Ficd%2Fentity%2F1183832314

[4] American Psychiatric Association. Prolonged Grief Disorder, DSM-5-TR educational guidance.
https://www.psychiatry.org/patients-families/prolonged-grief-disorder

[5] Substance Abuse and Mental Health Services Administration (SAMHSA). Coping with Bereavement and Grief.
https://www.samhsa.gov/communities/coping-bereavement-grief

[6] Canadian Psychological Association. Psychology Works: Grief in Adults.
https://cpa.ca/psychology-works-fact-sheet-grief-in-adults/

Pre-publication checklist

- Clinical reviewer checks grief, depression, PTSD, and prolonged grief descriptions and flags any claims requiring adjustment.
- Safeguarding reviewer checks suicide-risk language, child-specific guidance, and exercises for potential distress.
- Rwanda referral coordinator verifies contact details, availability, geography, hours, and referral permissions.
- Kinyarwanda and French translators complete meaning-based translation with local professional validation.
- Accessibility reviewer checks reading level, layout, screen-reader compatibility, and low-data alternatives.
- Content owner records reviewer names, dates, version, and next review date.

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